



P06 · 36 PAGES · 5 PHASES

The C-Section Recovery Planner

A five-week phased recovery planner built specifically for c-section postpartum , not adapted from a vaginal-birth template. Daily care logs, an incision check list, a medication tracker, a 6-week appointment prep page, and a partner handoff page. Heavy disclaimers throughout. not medical advice.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you start

A C-section is major abdominal surgery. Most postpartum planners ignore this entirely, they assume you walked out of the hospital. You did not.

This planner is built around what your body is actually doing in weeks 1 through 5, and what's realistic to do alongside it.

I'm not a doctor. I'm a person who had one c-section, planned for it badly the first week, planned for it better the next four, and made this so you wouldn't have to figure it out at 3am like I did.

This planner is for tracking, planning, and reflecting. **It is not medical advice.** Anything that feels physically wrong, call your OB. If something feels wrong, it usually is.

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Always follow your surgeon's specific instructions. Each c-section recovery is different. This is a general framework based on common recovery timelines, not a prescription. When in doubt, call your provider.

How to use this planner

- 36 pages total, undated.
- 5 phased sections (Weeks 1, 2, 3, 4, 5–6).
- Each phase has: what's happening in your body (general framework), realistic activity guidance, a daily self-care log, a mood + symptom tracker, and weekly reflection.
- Plus: medication tracker, incision-care log, mental-health watch, and a partner-handoff page.
- Use what helps. Skip what doesn't.

WEEK 1, The Hardest Week

What's generally happening

This is the week your body has just finished major surgery. Internal stitches are fresh. The incision is closed but tender. Your uterus is contracting back down (the cramps you feel, that's it). You're bleeding (lochia). You're also probably leaking milk, or struggling with milk supply, depending on feeding.

Sleep is fragmented. Pain medication is on a schedule. Walking is slow. Even standing up takes thought.

Your realistic activity guidance (THIS WEEK ONLY)

- Lift nothing heavier than the baby. Period.
- Walk: short, slow distances. The hallway. To the bathroom. To the couch.
- Driving: not yet (your surgeon will clear you, usually after 2 weeks, possibly later).
- Stairs: minimize. Set up a downstairs station.
- Showering: yes (once your surgeon clears), don't let water hit the incision directly. pat dry, don't rub.
- Sex / penetrative anything: no. (Most surgeons say wait 6 weeks for any of this.)

Daily care log

One page per day for 7 days. The fields below repeat on each daily page.

DAY ____ / DATE _____

PAIN LEVEL (circle): 1 2 3 4 5 6 7 8 9 10

PAIN MEDICATION TIMES (track to avoid gaps):

INCISION CHECK:

- ☐ Clean, dry
- ☐ Slight redness (normal first week)
- ☐ Increasing redness or warmth, CALL OB
- ☐ Pus or discharge, CALL OB IMMEDIATELY
- ☐ Fever above 100.4, CALL OB IMMEDIATELY

LOCHIA / BLEEDING:

- ☐ Light
- ☐ Moderate
- ☐ Heavy
- ☐ Saturating a pad / hr, CALL

NOTES (one line, what felt different today):

ONE THING I DID FOR MYSELF (no matter how small):

ONE THING MY PARTNER / SUPPORT PERSON DID:

Mental-health watch (read once at start of week 1)

The baby blues affect 80% of new birthing parents and usually resolve by day 14. The signs:

- Crying more than usual
- Feeling overwhelmed
- Sleep-deprived irritability
- Worried about the baby

These are normal. They should ease.

What's NOT just baby blues:

- Thoughts of harming yourself or the baby
- Feeling "numb" or "nothing" toward the baby for days
- Inability to sleep when the baby sleeps because you can't stop checking
- Intrusive scary thoughts that won't go away
- Sense of "this isn't going to get better"

These are signs of postpartum depression or postpartum anxiety. **Call your OB or therapist.** Don't wait. PPD/PPA are common (1 in 7) and treatable.

US Maternal Mental Health Hotline: **1-833-852-6262** (24/7, free, confidential)

End-of-week 1 reflection

The hardest moment this week was: _____

The thing my body did that surprised me: _____

What helped most: _____

What didn't help (don't let anyone repeat this): _____

Going into week 2, I'd like: _____

WEEK 2, Slightly More Functional

What's generally happening

Week 2 is usually noticeably better than week 1. Pain is improving. You can usually transition from prescription pain meds to whatever over-the-counter option your OB recommends. The incision is healing externally. internal healing is still happening (and takes much longer).

You might be tempted to do more. **Don't do more.** Most c-section setbacks happen in week 2-3 because people feel "better" and lift / drive / exercise too early.

Activity guidance for week 2

- Still: no lifting more than the baby
- Walking distances can increase, but slowly
- Driving: still no (most surgeons say wait 2 weeks minimum, sometimes longer)
- Stairs: a couple of times a day is okay. minimize
- Light household activity: putting dishes away (don't lift heavy ones), wiping the counter, folding laundry sitting down
- Still no: vacuuming, mopping, anything bending-and-lifting
- Sex / penetrative: still no

Daily care log (same template as week 1, 7 pages)

DAY ____ / DATE ____

PAIN LEVEL: 1-10

INCISION CHECK

LOCHIA

NOTES

ONE THING I DID FOR MYSELF

ONE THING MY PARTNER DID

Watch list for week 2

If you have any of these, call your OB:

- Increasing pain at the incision (not decreasing)
- Redness spreading, warmth, swelling at incision
- Pus or unusual discharge
- Fever 100.4+
- Heavy bleeding (soaking pad in <1 hr) or large clots
- Chest pain, shortness of breath, leg pain/swelling (blood clot signs, emergency room)
- Severe headache that doesn't respond to medication
- Mental-health changes from last week's watch list

End-of-week 2 reflection

This week I noticed: _____

The activity I tried that I shouldn't have: _____

The activity I avoided that I could have done: _____

How I'm thinking about week 3: _____

WEEK 3, The “I Should Be Fine By Now” Trap

What’s generally happening

Week 3 looks like everyone expects week 1 to look. You’re walking around. You feel mostly okay. The pain is way better. Your friends are texting “how are you” again because the casseroles stopped.

This week is a trap. Hormones drop again around day 14–21 (estrogen and progesterone both). Energy looks better than it is. Internal healing is still very much ongoing.

Activity guidance for week 3

- You can probably drive now if your OB has cleared you
- You can do more walks, short outdoor walks
- Light cooking, light cleaning at sitting level
- You can probably leave the house briefly with the baby
- Still NO: heavy lifting (above the baby), running, ab exercises, anything bouncing, sex

Daily care log (same template, 7 pages)

Mental-health pulse check

This is when delayed-onset postpartum depression can show up. If you’ve made it through weeks 1–2 thinking “okay, I’m fine,” and now you’re slipping ,

- Track your mood daily this week. Pattern it.
- If 3+ days in a row of low mood, anxiety, or detachment, call.

End-of-week 3 reflection

What I assumed I'd be doing by now that I'm not: _____

What I AM doing that I'm proud of: _____

What I need more of: _____

What I need less of: _____

WEEK 4, Establishing the New Normal

What's generally happening

By week 4, the external incision is mostly closed and healing into a scar. Internal healing is still ongoing (full internal recovery takes longer than the surface suggests). Your body is starting to settle into a postpartum baseline that isn't going to look like your pre-pregnancy baseline for a while.

Many partners go back to work this week (if they had paternity leave). The first week alone-with-the-baby happens here. This can be hard. Even if you've been "ready", first day solo is its own thing.

Activity guidance for week 4

- Walks can be longer (30–45 min)
- You can pick up the baby + their carrier (about 15 lbs total, most surgeons clear this)
- Still no: lifting toddlers, heavy groceries, gym workouts, ab exercises, running, sex (most OBs still say wait until 6-week clearance)

The “first day solo” page

THE PLAN FOR THE FIRST DAY ALONE WITH THE BABY:

What time my partner leaves: _____

What time my partner returns: _____

The minimum I'm trying to do today (write it small):

The person I will text if I need help: _____

What I have set out at arm's reach:

- ☐ Water bottle (full)
- ☐ Snacks (one-handed: granola bar, fruit, etc.)
- ☐ Phone with charger
- ☐ Diaper supplies
- ☐ Bottle / pump (if applicable)
- ☐ A book or magazine for me (something to read while baby naps)

Daily care log (template, 7 pages)

End-of-week 4 reflection

The first solo day was: _____

What I learned about my own limits: _____

What I'd tell next-week-me: _____

WEEK 5-6, Pre-Clearance

What's generally happening

Most c-section moms have their 6-week postpartum appointment around this time. Your OB will check the incision, ask about pain, check your overall recovery, and typically clear you for full activity.

But “cleared” doesn’t mean “back to pre-pregnancy.” Cleared means “the immediate surgical recovery is done.” Your core is still healing. Pelvic floor work is real. Some people see a pelvic floor physical therapist starting around week 6, this is normal and helpful, especially after c-section.

Activity guidance for weeks 5-6

- Track how you feel before and after any new activity
- Listen to your body. If it hurts, stop.
- After 6-week clearance: gradual return to exercise. Walking → walking + light yoga → light strength → running last.
- Don't try to “bounce back.” Most full-body recovery takes 6-12 months minimum.

Your 6-week appointment prep page

QUESTIONS FOR MY OB AT THE 6-WEEK APPOINTMENT:

About the scar:

- ☐ Is it healing as expected?
- ☐ Any concerns about scar tissue / adhesions?
- ☐ When can I start scar massage?

About my body:

- ☐ Cleared to exercise? At what intensity?
- ☐ Pelvic floor physical therapy, should I see one?
- ☐ Diastasis recti, should I get checked?
- ☐ Birth control options (if relevant)
- ☐ Any concerns about diastasis recti recovery?

About mental health:

- ☐ Postpartum depression screening (EPDS or similar)
- ☐ How I've been feeling (be honest. this is the moment to flag if struggling)

About future pregnancies (if relevant):

- ☐ When can I safely conceive again?
- ☐ VBAC eligibility?
- ☐ Recommended interval between pregnancies?

Anything else I want to ask:

Final week reflection

6 weeks ago I had a c-section. Today I am: _____

The version of recovery I expected vs. what happened:

What I want my partner to keep doing: _____

What I want to do for myself going forward: _____

A note to a friend who's about to have a c-section: _

SUPPORTING PAGES

Medication tracker

[illegible]

Incision care log

Date | Appearance | Care done | Any concerns

[Repeat for 6 weeks]

Partner handoff page

Tear this out and tape it where your partner will see it. Or text them a photo.

What I cannot do this week (don't ask, don't suggest): - Lift anything heavier than the baby - Drive (until cleared) - Vacuum, mop, carry laundry baskets - Climb stairs more than a few times a day - Reach overhead, twist hard, bend deeply - Sex / penetrative (not until 6-week clearance)

What I need from you (do without asking): - Bring me water + snacks every 2 hours - Take ALL the night diaper changes (you can do these) - Bring the baby to me for nursing. you handle put-down - Take the laundry up and down stairs - Hold the line on visitors - Daily check-in: "What was the hardest moment today?" - One night this week: let me sleep 5+ hrs straight

Watch list, call my OB if I: - Have a fever 100.4+ - Have increasing pain at the incision - Have redness, pus, or warmth at incision - Bleed through a pad in under an hour - Have chest pain, shortness of breath, or leg pain/swelling - Stop being able to eat or drink - Say something that sounds like I might hurt myself or the baby

You will probably feel: - Tired - Overwhelmed - Like nothing you do is enough - Like you don't know how to help

These feelings are normal. The fact that you're trying is the point.

A LAST NOTE

You had major surgery. You're keeping a tiny human alive on no sleep. You did not "fail" if you needed a c-section, even if the birth wasn't what you planned.

Your scar is not a defect. It's the entrance through which your kid came into the world.

The recovery is slower than anyone tells you. You will probably feel mostly normal by week 6 and not actually 100% for a year. That's also fine. Bodies take time.

If something feels wrong, physically or mentally, call. Always call.

You're doing the thing.

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This planner is a general framework and tracking tool. It is not medical advice. Every c-section recovery is individual, and your OB / surgeon is the authority on your specific recovery. If something feels wrong, contact your provider immediately. For mental-health concerns, contact your provider, the US Maternal Mental Health Hotline at 1-833-852-6262, or the Suicide & Crisis Lifeline at 988.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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